3.13 Acute Respiratory Failure

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • Confused, does not answer questions **Objective** • PaO_2 50 mmHg, $PaCO_2$ 65 mmHg, pH 7.24 • RR 8/min and shallow	**ACTUAL** **Impaired spontaneous ventilation** related to respiratory muscle fatigue as evidenced by hypercapnia, acidosis and bradypnea.	Within 2 hours ventilation is adequate and ABG is improving.	• Call for urgent medical help. • Open the airway and support breathing with a bag-valve-mask if needed. • Get ready for non-invasive ventilation or intubation. • Watch ABG, SpO_2, HR and level of consciousness.	pH and $PaCO_2$ getting better: **goal met**.
Subjective • Family say • "He is not making sense." **Objective** • GCS 10 • Restless and disoriented	**ACTUAL** **Acute confusion** related to hypoxemia and hypercapnia as evidenced by disorientation and reduced GCS.	Becomes oriented as gas exchange improves.	• GCS and orientation every hour. • Bed low, padded side rails as policy allows. • Reorient him often, using simple words. • Correct the hypoxemia and hypercapnia as prescribed.	Oriented: **goal met**.
Subjective • *No specific complaint* **Objective** • Not fully conscious • Cough is weak	**RISK** **Risk for aspiration** as evidenced by risk factors: decreased level of consciousness and impaired cough.	Airway stays clear.	• Head of the bed up at 30–45 degrees. • Have suction ready. • Nothing by mouth until it is safe for him to swallow.	No aspiration: **goal met**.